



MENTAL HEALTH RESOURCES

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Differential Diagnosis of Child Abuse

The differential diagnosis for severe attachment pathology (i.e., a child rejecting a parent) is child abuse by one parent or the other.

1) Path A – Targeted Parent Abusive:

either the targeted parent is abusing the child, creating the child's attachment pathology toward that parent (a 2-person attribution of causality),

2) Path B – Allied Parent Abusive:

or the allied parent is psychologically abusing the child by creating a shared persecutory delusion and false attachment pathology imposed on the child for the secondary gain of manipulating the court's decisions regarding child custody, and to meet the parent's own emotional and psychological needs (3-person attribution of causality).

In all cases of severe attachment pathology (i.e., a child rejecting a parent), a proper risk assessment for possible child abuse needs to be conducted to the appropriate differential diagnosis for each parent.

Diagnostic Questions to be Answered

These are the diagnostic questions that need to be answered:

Handout: [Differential Diagnostic Questions \(https://drcachildress-consulting.com/wp-content/uploads/2019/11/Differential-](https://drcachildress-consulting.com/wp-content/uploads/2019/11/Differential-)

Diagnostic-Questions-1-26.pdf

Diagnosis Series: Diagnosing Attachment Pathology

Dr. C. A. Childress



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Shared Persecutory Delusion

The diagnostic concern is a possible shared persecutory delusion created by the pathogenic parenting of the allied parent.

From the APA: “Persecutory Type: delusions that the person (or someone to whom the person is close) is being malevolently treated in some way” (American Psychiatric Association, 2000)

Google malevolent: Having or showing a wish to do evil to others

The differential diagnosis of a possible shared persecutory delusion should be anticipated surrounding court-involved custody conflict and attachment pathology displayed by the child.

From Walters & Friedlander: “In some RRD families [resist-refuse dynamic], a parent’s underlying encapsulated delusion about the other parent is at the root of the intractability (cf. Johnston & Campbell, 1988, p. 53ff; Childress, 2013). An encapsulated delusion is a fixed, circumscribed belief that persists over time and is not altered by evidence of the inaccuracy of the belief.”

From Walters & Friedlander: “When alienation is the predominant factor in the RRD [resist-refuse dynamic}, the theme of the favored parent’s fixed delusion often is that the rejected parent is sexually, physically, and/or emotionally abusing the child. The child may

come to share the parent's encapsulated delusion and to regard the beliefs as his/her own (cf. Childress, 2013)." (Walters & Friedlander, 2016, p. 426)

Walters, M. G., & Friedlander, S. (2016). When a child rejects a parent: Working with the intractable resist/refuse dynamic. *Family Court Review*, 54(3), 424–445.

A proper assessment for possible delusional thought disorder pathology in the family needs to be conducted. The assessment for delusional thought disorder pathology is a Mental Status Exam of thought and perception (frontal lobe executive function systems for linear-logical reasoning) as described by Martin (1990):

Chapter 207 of Clinical Methods: Mental Status Exam
(<https://www.ncbi.nlm.nih.gov/books/NBK320/>).

From Martin: "The inability to process information correctly is part of the definition of psychotic thinking. How the patient perceives and responds to stimuli is therefore a critical psychiatric assessment. Does the patient harbor realistic concerns, or are

these concerns elevated to the level of irrational fear? Is the patient responding in exaggerated fashion to actual events, or is there no discernible basis in reality for the patient’s beliefs or behavior?”

Q: Does the child harbor realistic concerns, or are these concerns elevated to the level of an irrational fear?

Q: Is the child responding in exaggerated fashion to actual events?

Q: Is there no discernible basis in reality for the child’s beliefs and behavior?

From Martin: “Of all portions of the mental status examination, the evaluation of a potential thought disorder is one of the most difficult and requires considerable experience. The primary-care physician will frequently desire formal psychiatric consultation in patients exhibiting such disorders.”

Diagnosis Series - Chapter 4: Diagnosing a Persecutory Delusion
Dr. C. A. Childress



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Diagnostic & Symptom Instruments

Diagnostic Checklist

This instrument documents the child's symptoms in three areas, 1) Attachment Bonding Suppression, 2) Narcissistic Personality Traits, and 3) a Persecutory Delusion.

Checklist Handout: [Diagnostic Checklist for Pathogenic Parenting](https://drcachildress-consulting.com/wp-content/uploads/2019/11/Diagnostic-Checklist-1-20.pdf) (<https://drcachildress-consulting.com/wp-content/uploads/2019/11/Diagnostic-Checklist-1-20.pdf>).

Parenting Practices Rating Scale

This instrument documents the parenting practices of the targeted parent as either abuse-range or normal-range.

Checklist Handout: [Parenting Practices Rating Scale](https://drcachildress-consulting.com/wp-content/uploads/2019/11/Parenting-Practices-Rating-Scale-ver-3-2-14-17.pdf) (<https://drcachildress-consulting.com/wp-content/uploads/2019/11/Parenting-Practices-Rating-Scale-ver-3-2-14-17.pdf>).

Parent-Child Relationship Rating Scale

This instrument is an outcome measure of daily reports to the family therapist by the parent on three areas of functioning, 1) Affection, Cooperation, and Social Involvement.

Outcome Measure: [PC-RRS](https://drcachildress-consulting.com/wp-content/uploads/2019/11/PC-RRS.pdf) (<https://drcachildress-consulting.com/wp-content/uploads/2019/11/PC-RRS.pdf>).

YouTube: 12 Associated Clinical Signs (ACS)

Introduction to the 12 Associated Clinical Signs
Dr. C. A. Childress



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ACS 1: Use of the Word "forced" ACS 2: Empowering the Child to Reject

ACS 4: Parental Replacement ACS 5: Unforgivable Event

ACS 6: "Liar" - "Fake" ACS 7: Themes for Rejection ACS 8: Unwarranted Use of Word "abuse"

ACS 9: Excessive Texting & Email ACS 10: Role Reversal Use of the Child ACS 11: Deserves to Be Rejected

ACS 12: Disregard of Court Authority

Resource for Professional Reading and Citation

Annotated References for Foundations (Childress, 2015).
(<https://drcachildress-consulting.com/wp-content/uploads/2019/11/Annotated-Foundation-References-2026.pdf>).